INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly cetuximab + irinotecan

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00330
Version 6

> Cetuximab on Day 1 and Day 8; irinotecan on Day 1. Repeat the reference cycle every 14 days while beneficial and tolerable. There is no fluorouracil home pump.

Treatment stages

01 **A 14-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks growth messages at the cell surface

Cetuximab attaches to EGFR on the outside of cells and blocks some growth messages. In this bowel-cancer plan, validated extended RAS testing helps identify whether treatment is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts deoxyribonucleic acid (the cell's genetic material) (DNA) copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

Why this option?

This combination is used for RAS wild-type bowel cancer that has spread. This NCCP pathway is a second-line option after oxaliplatin-based treatment has failed or cannot be used. Your team explains the aim, alternatives and how benefit will be reviewed.

A plan that can change

Discuss how treatment aims to control the cancer and symptoms, and whether a response could allow a further local treatment in your individual case. Review benefit, side effects and your priorities as the plan changes.

EPIC: extended RAS analysis

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Adding cetuximab to irinotecan improved tumour shrinkage and the time before cancer worsened in a selected trial group. Overall survival was similar between the groups. Many patients in the comparison group later received cetuximab. The trial used a different irinotecan schedule from this NCCP plan.

CHAPTER 02

Your schedule, tests and appointments

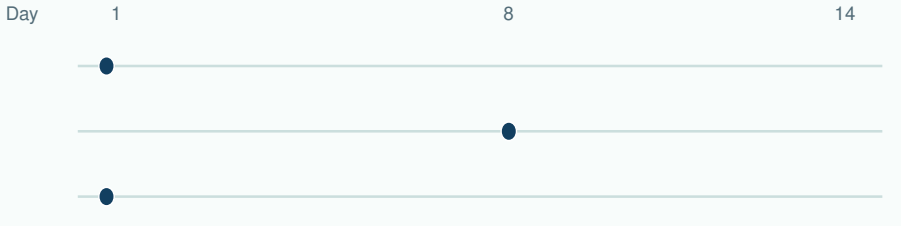
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, recovery and review

Weekly cetuximab + irinotecan

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 14-day reference cycle

D1 D8

Cetuximab on Day 1 and Day 8; irinotecan on Day 1. Repeat the reference cycle every 14 days while beneficial and tolerable. There is no fluorouracil home pump.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00330 v6 · official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Weekly cetuximab + irinotecan: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab on Day 1 and Day 8; irinotecan on Day 1. Repeat the reference cycle every 14 days while beneficial and tolerable. There is no fluorouracil home pump.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00330	Baseline blood count, kidney and liver profile	Review results before treatment	Regular blood count, kidney and liver tests as arranged	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium NCCP 00330	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00330	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration NCCP 00330	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day: act at the first liquid stool using your supplied rescue plan and electrolyte fluids; contact the team early.	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Cancer response and treatment choices NCCP 00330	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1 **A 14-day reference cycle**

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Visits, access and care between treatments

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Ask whether a peripheral cannula or an existing line is appropriate; a port is not automatically required.

STEP 2

At the treatment unit

The first weekly cetuximab infusion takes about two hours; later weekly infusions usually take about one hour. Irinotecan usually takes about 90 minutes. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

There is no fluorouracil home pump and no disconnection visit in this regimen.

STEP 4

Between doses

Attend the separate Day 8 cetuximab-only visit. This is not another irinotecan or oxaliplatin day. Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

Preparing access

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

The exact delivery plan

Cetuximab on Day 1 and Day 8. Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Radiotherapy relationship

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab infusion	 **Day 1** intravenous (IV)
Cetuximab: second visit	 **Day 8** IV
Day-ward chemotherapy	 **Day 1** IV
Skin, nails and minerals	 **Days 1–14** Can build over successive cycles
Early cramps / sweating	 **Day 1** Tell the nurse
Delayed diarrhoea	 **Days 2–14** Can occur after 24 hours
Blood counts / infection risk	 **Days 1–14** Blood tests and symptoms guide review
Energy and bowel symptoms	 **Days 1–14** Variable timing

Your planned visits: Cetuximab on Day 1 and Day 8. Irinotecan is given on Day 1; no home fluorouracil pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Care continues: Continue the skin-care plan and mineral checks arranged after cetuximab. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

Why these bands stay broad: NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00330 v6 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

Day 2–7: Care between visits

Follow the skin-care or bowel-symptom plan provided. There is no pump to remove. Call early for symptoms; do not wait for the next appointment.

Day 8: Cetuximab-only visit

Premedicines and observation still apply. Attend the planned review and infusion.

Day 9–14: Between visits / next review

Record symptoms and follow your support plan. Call if unwell; do not wait for the next cycle.

Support around the medicines

1 Skin, nails and minerals

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

2 Bowel symptoms and fluids

Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice. At the first liquid stool, start your individually supplied diarrhoea rescue plan and drink electrolyte-containing fluids as instructed. Contact your team early; do not wait for severe diarrhoea. If you do not have a rescue plan, contact the team immediately for instructions.

3 Comfort and daily life

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

4 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

5 The cetuximab-only visit

Day 8 contains cetuximab only. Its premedicines and observation still matter, but the chemotherapy-day anti-sickness course is not automatically repeated.

Symptoms in context

Report what changed and how it affects you.

Living with treatment

Use the support available.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

Will I have a pump at home?

There is no home fluorouracil pump in this regimen. Your team chooses a cannula or central line based on your veins and wider treatment plan.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

Why is RAS testing mentioned?

RAS testing selects cetuximab treatment in colorectal cancer. It is not the selection test for the separate head-and-neck radiotherapy indication. Confirm the cancer-specific plan with your team.

> **Try explaining the plan back:** Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Irinotecan

Emetogenic risk: Moderate

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. Use with caution in asthma; review suitability before prescribing. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · weekly cetuximab only

Active medicines: Cetuximab

Emetogenic risk: Low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Cetuximab premedication remains applicable.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab Reconcile low-risk antiemesis with this corticosteroid; do not duplicate steroids.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed No automatic delayed steroid course is carried over from Day 1 chemotherapy.

Additional phase-specific support

- Cetuximab only: no irinotecan, oxaliplatin, fluorouracil or atropine is scheduled on Day 8.
- Continue skin care, electrolyte monitoring, infusion precautions and the urgent contact plan.
- Low-risk antibody-only antiemesis is distinct from Day 1 moderate-risk chemotherapy prophylaxis.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes

Cetuximab

Eye pain or new visual change needs prompt advice.

2 Mouth / swallowing

Treatment

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

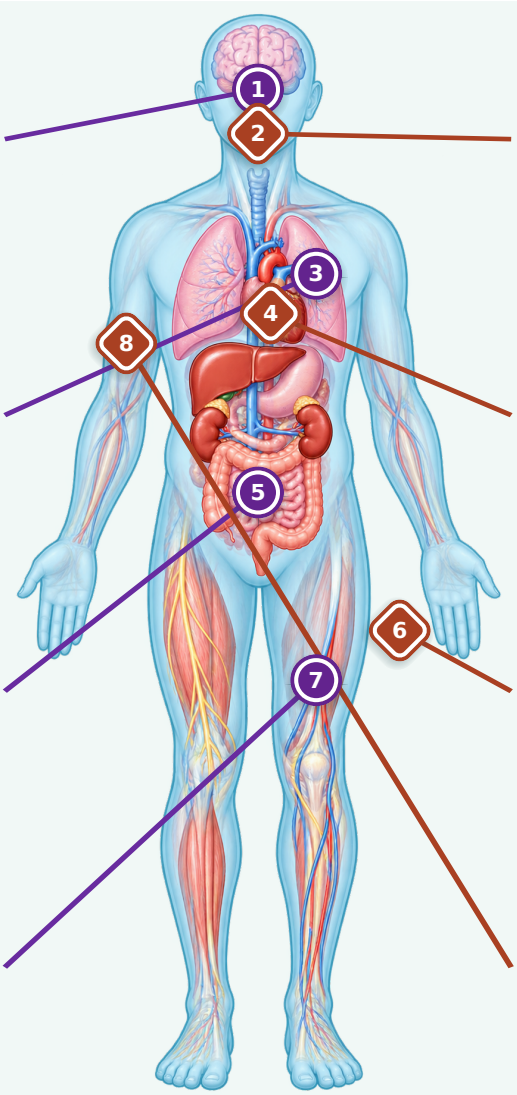
Cetuximab

New breathing difficulty requires prompt assessment.

4 Chest

Treatment

New chest pain needs urgent assessment.



5 Bowel

Irinotecan

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Cetuximab

Report painful rash, cracks or infected skin early.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Infusion access

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00330 v6 · official regimen**
   https://healthservice.hse.ie/documents/6785/330_v6_Cetuximab_7days_irinotecan_14_day.pdf
2. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
3. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
4. **EPIC extended RAS primary report**
   https://academic.oup.com/oncolo/article/26/2/e261/6445358
5. **NCCP NCIS supportive antiemetic medicines V8 (2025)**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq